



Empacoza

Beyond Glycemic Control

Cardio-Renal-Metabolic Protection for the complex Type 2 Diabetes patient landscape.



The *hidden burden* of Type 2 Diabetes

Type 2 Diabetes is a multi-systemic disease that extends far beyond glucose levels.

Heart

Increased risk of cardiovascular death and structural heart failure.

Glucose

The clinical necessity for early and durable glycemic control.



Kidney

Chronic hyperfiltration leading to irreversible nephron loss and CKD.

Metabolic

Persistent weight management challenges and insulin resistance.

Targeting the *SGLT2 mechanism* for systemic protection

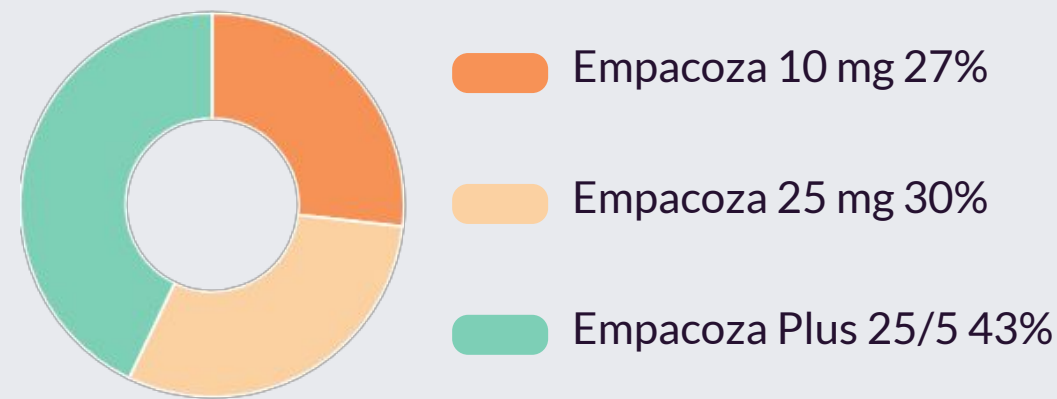
A scientific approach to glucose excretion and hemodynamic stability through SGLT2 inhibition.

Empacoza inhibits Sodium-Glucose Co-Transporter 2 (SGLT2) in the proximal convoluted tubule, fundamentally altering the patient's metabolic trajectory. This mechanism facilitates the excretion of approximately 78g of glucose per day through the urine. Beyond glucose, the reduction in sodium reabsorption leads to osmotic diuresis and natriuresis, which lowers both preload and afterload. This dual action provides the metabolic relief and hemodynamic "offloading" necessary to protect vulnerable organ systems.

Empacoza delivers *potent and durable* HbA1c reduction

Robust glycemic control as the foundation for multi-organ protection.

HbA1c Reduction by Therapy

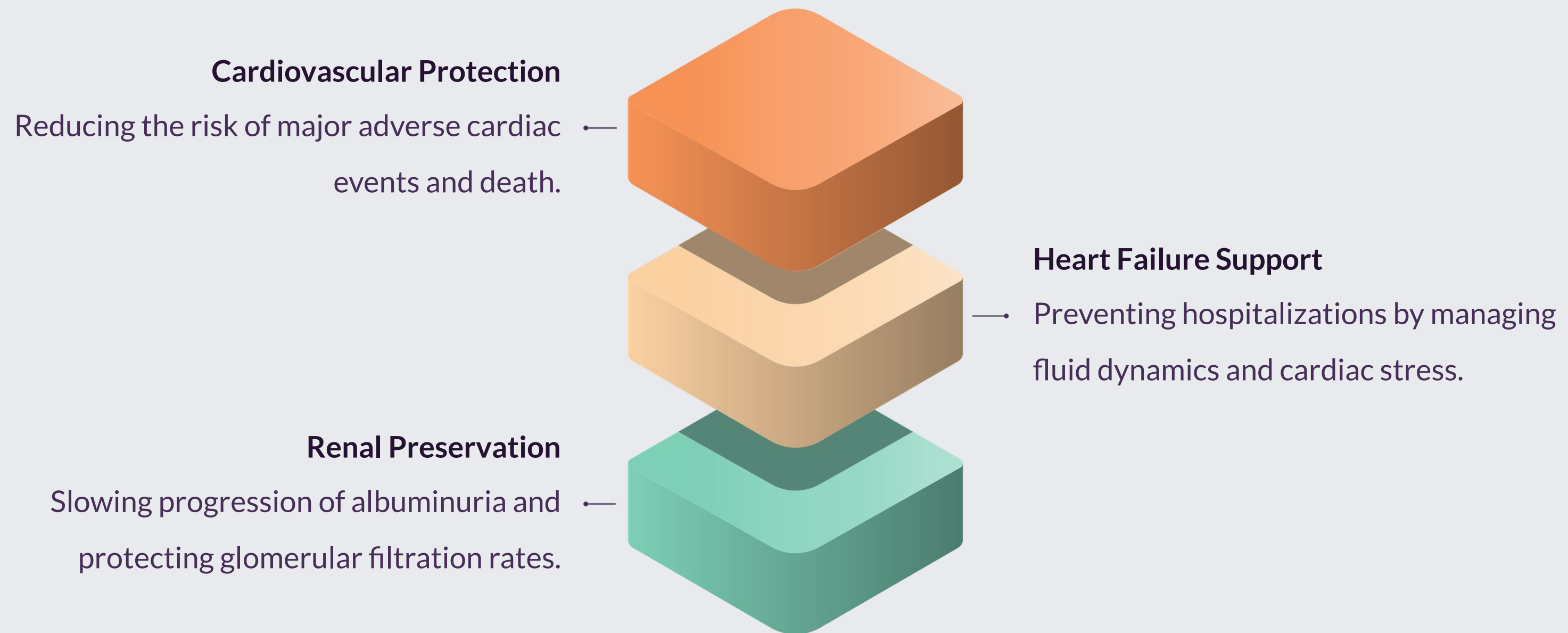


Clinical Study Data

Therapy Baseline	HbA1c Reduction	Weight Change
Empacoza 10 mg	-0.7% to -0.8%	-2.5 kg
Empacoza 25 mg	-0.8% to -0.9%	-2.8 kg
Empacoza Plus 25/5	-1.1% to -1.3%	-3.1 kg

Moving *beyond HbA1c* to the CRM protection triad

Defining a new standard of care through three clinical pillars of organ safety.



Proven 38% *reduction* in cardiovascular death

The landmark outcome that redefined SGLT2 inhibitors as life-saving therapy.

38 % ↘

CV Death Risk

Observed early in therapy and sustained throughout the treatment period.

2.4 M ↗

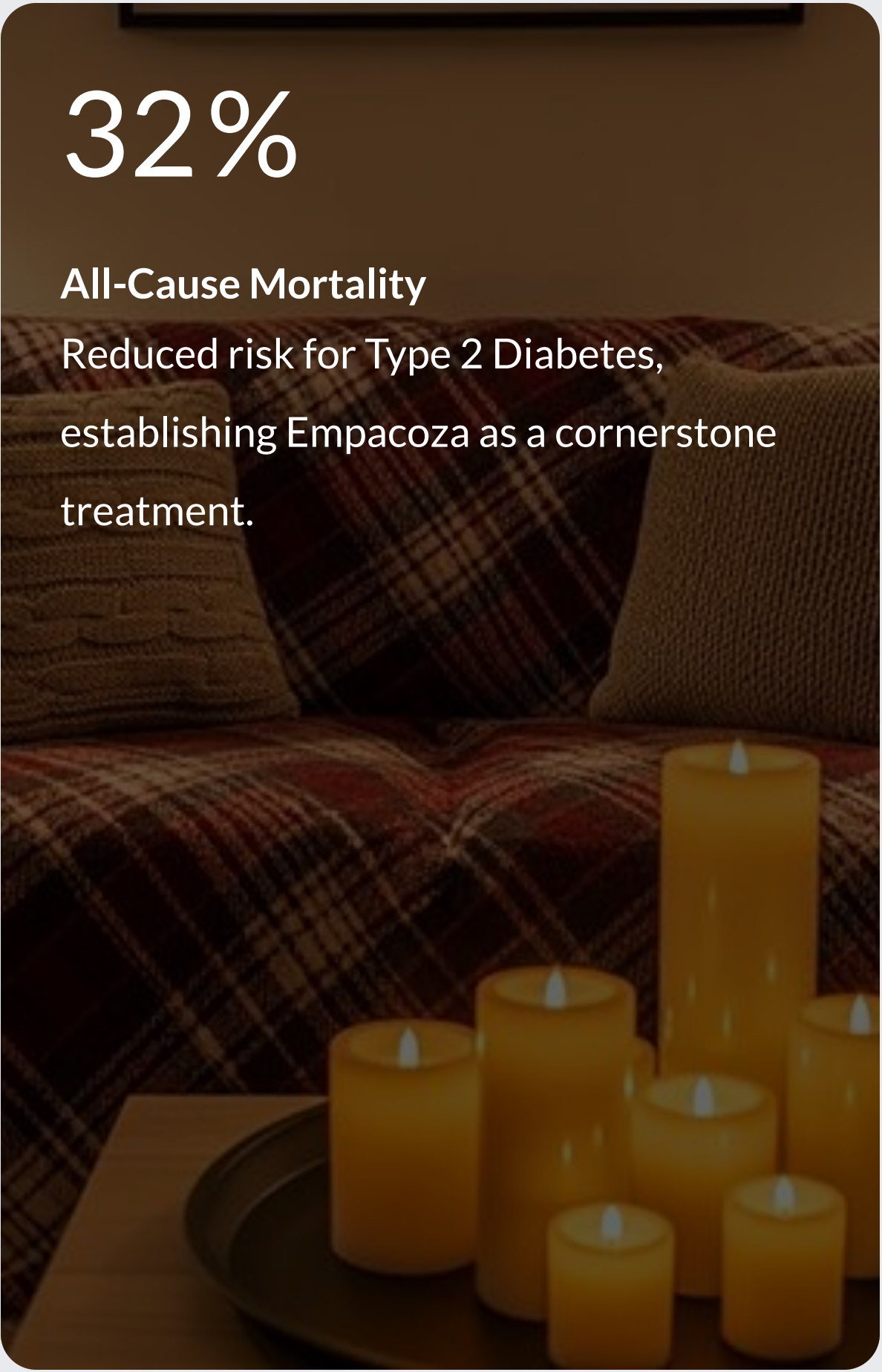
Inspiring Heading

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32%

All-Cause Mortality

Reduced risk for Type 2 Diabetes, establishing Empacoza as a cornerstone treatment.



Reducing the risk of *Heart Failure* hospitalizations

Heart failure is a frequent complication of T2DM. Empacoza acts on hemodynamic drivers, reducing hospitalization risk by **35%**. By promoting natriuresis and reducing plasma volume without sympathetic nervous system stimulation, it stabilizes the diabetic heart. This benefit persists across patients regardless of history, making it a vital clinical tool.

Preserving *renal function* at the nephron level

Delaying the progression of diabetic kidney disease through hemodynamic regulation.

Sustained eGFR

Significant slowing of the annual decline in renal function.

Hard Outcomes

Lowering the composite risk of doubling serum creatinine or requiring dialysis.



Albuminuria Control

Reduced risk of progression to macroalbuminuria by 38%.

The *Empacoza* portfolio: Precision dosing for every patient

Empacoza 10 mg

The standard starting dose for broad CRM protection and glycemic control.

Empacoza 25 mg

Higher potency for patients requiring more intensive HbA1c and weight management.

Empacoza Plus (25/5 mg)

The fixed-dose combination of Empagliflozin and Linagliptin for synergistic dual-mechanism control. All formulations are designed for once-daily convenience to improve patient adherence.

Empacoza Plus: Synergy of dual-pathway inhibition

Combining SGLT2 and DPP-4 inhibition in a single, powerful tablet.

Empagliflozin (SGLT2i)

- ✓ **Primary Site: Renal Proximal Tubule**
- ✓ **Glucose Effect: Insulin-independent excretion**
- ✓ **Side Benefit: Weight and BP reduction**
- ✓ **Superior HbA1c reduction
compared to component alone**

Linagliptin (DPP-4i)

- ✓ **Primary Site: Incretin (GLP-1/GIP)**
- ✓ **Glucose Effect: Glucose-dependent secretion**
- ✓ **Side Benefit: Weight-neutral safety**
- ✓ **Superior HbA1c reduction
compared to component alone**

Identifying the *ideal patient* for Empacoza therapy

Strategic patient profiling to maximize clinical outcomes and protection.

The High CV-Risk Patient

Established CVD where reducing death is the primary clinical objective.

The Uncontrolled A1c

Patients failing on Metformin who require potent, weight-positive secondary therapy.



The Heart Failure Patient

Patients at risk for or currently managing fluid overload and cardiac strain.

The Renal-First Patient

Patients with declining eGFR who need nephroprotection alongside glucose control.

Safety

Maintaining *safety* *and tolerability* in daily practice

A straightforward monitoring dashboard for healthcare professionals.

01

Hydration

Advise patients to maintain adequate fluid intake to support osmotic diuresis.

02

Renal Monitoring

Assess renal function (eGFR) at baseline and periodically during treatment.

04

DKA Awareness

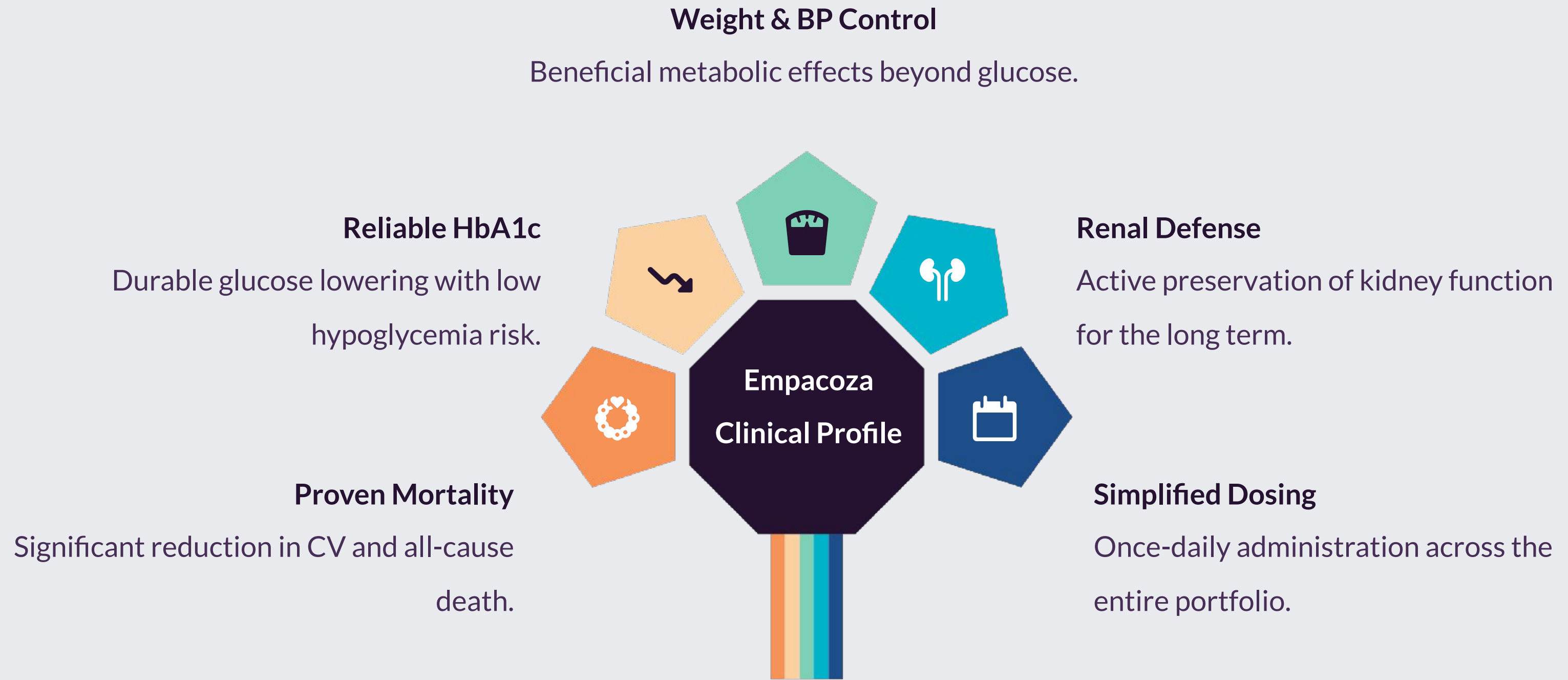
Educate on symptoms of euglycemic ketoacidosis, particularly during illness or surgery.

03

Genital Hygiene

Counsel on proper hygiene to mitigate the risk of common mycotic infections.

Why Empacoza? Five clinical reasons to choose protection



Control. Protect. Simplify.

The path forward.

Consolidating the clinical strategy for superior Type 2
Diabetes management.

Glycemic Control

Move beyond the limitations of traditional
glucose-centric models.

Organ Protection

Maximize the patient's lifespan
through comprehensive care.

Treatment Simplification

Simplify the physician's workflow
for a higher standard of care.



Treat Beyond HbA1c with Empacoza

Empowering physicians to protect the heart, kidneys, and metabolic health of their patients.

Choose Empacoza for comprehensive cardio-renal-metabolic protection. Zeta Pharma: Committed to advancing T2DM care.